MICHIGAN MEDICINE
UNIVERSITY OF MICHIGAN

Pemivibart (Pemgarda) Step-by-step

Pemivibart is a 1-hour infusion with a 2-hour observation period, given every 3 months, for pre-exposure prophylaxis against COVID-19 infection in high-risk patients. Anaphylaxis and infusion-related reactions can occur. Anaphylaxis has been observed with pemivibart in 0.6% (4/623) of participants in a clinical trial, and hypersensitivity reactions in 4% of patients.

This is most likely to benefit those patients at highest risk for severe disease, and/or patients unlikely to mount an immune response to prior infection and/or vaccination. Use of pemivibart does not replace vaccination against COVID-19. Pemivibart dose should be delayed for 2 weeks post COVID-19 vaccination.

Suggested potential use cases:

1. *Lung transplant recipients*
2. *B-cell depletion for malignancy*

Step-by-step Process for ordering Pemivibart (Pemgarda):

1. Physician and high-risk patient have a risk/benefit discussion
2. If patient is interested in treatment, provider places "**Referral order for pemivibart**"
3. Order sent directly to the **COVID mAb pharmD** inbasket.
4. Trained PharmD calls the patient, discusses the treatment, obtains and documents verbal consent of EUA medication, and provides the patient with the FDA EUA Fact Sheet for patients
5. PharmD places therapy plan for pemivibart (for every 3 months) for co-sign by referring physician
6. Physician receives the order for review and to cosign in MiChart inbasket
7. PharmD routes therapy plan to MIST for insurance review
8. If pemivibart is covered by the patient's insurance, MIST will schedule the patient.
9. If pemivibart is not covered by the patient's insurance, the Infusion Business Team will first reach out to the provider with any options (appeal, peer-to-peer) if any.
    a. If the provider would still like to move forward with the treatment, the Infusion Business Team will contact the patient with an estimated cost of the infusion.
    b. If the patient would like to move forward, MIST will connect them with a Patient Financial Counselor and/or our Infusion Patient Assistance to see if there are any programs that the patient would qualify for to cover the treatment. If none, patient would be responsible to pay out of pocket for the service. If the patient does not want to move forward with the treatment, MIST will send an inbasket note to the ordering provider and nursing inbasket.
10. Patients will be scheduled for every 3 months, and after 12 months, therapy plan will be sent to referring provider to review and approve, or to discontinue.

Last revised: 07/2024	**Antimicrobial Stewardship Subcommittee:** N/A	**P&T:** N/A
References		
	Citation	
Revision History		
Date	**Revision**	

The recommendations in this guide are meant to serve as treatment guidelines for use at Michigan Medicine facilities. If you are an individual experiencing a medical emergency, call 911 immediately. These guidelines should not replace a provider's professional medical advice based on clinical judgment, or be used in lieu of an Infectious Diseases consultation when necessary. As a result of ongoing research, practice guidelines may from time to time change. The authors of these guidelines have made all attempts to ensure the accuracy based on current information, however, due to ongoing research, users of these guidelines are strongly encouraged to confirm the information contained within them through an independent source